GROWTH HORMONE & ANABOLIC SIGNALING

CJC-1295 — the full review

GROWTH HORMONE-RELEASING HORMONE ANALOGUE

What it is

CJC-1295 is a lab-made analogue of the first 29 building blocks of a natural brain hormone, growth hormone-releasing hormone (GHRH), modified with several amino-acid substitutions so it resists breakdown longer than the natural signal. It is sold under one name but actually exists as two chemically distinct products: “with DAC,” which carries an added chemical anchor that binds permanently to a protein in the blood, and “without DAC” (also sold as Mod GRF 1-29), which does not.

That distinction matters more than almost anything else about this compound. The with-DAC form remains active in the body for roughly a week per dose and is the form used in essentially all of the published human research. The without-DAC form clears within minutes and has never been evaluated in a controlled human study of any kind.

CJC-1295 has never been approved by the FDA, in either form, for any use, in any country. It is sold today only as a compounded preparation, and it frequently appears alongside other growth hormone-releasing peptides in combination blends.

With DAC vs. without DAC

	WITH DAC	WITHOUT DAC
Half-life	Roughly 6–8 days, anchored to the blood protein albumin	Never measured in a human study; the commonly quoted ~30-minute figure has no published human source
Human studies	Three published studies, all healthy volunteers, a few dozen people total	Zero controlled human studies of any kind
Effect on GH/IGF-1	Raises baseline growth hormone levels for days at a time; measured and published	Not measured in humans
The only efficacy trial	Enrolled 120 people (HIV-associated visceral obesity), terminated early in 2006 after a participant died, results never published	No efficacy trial has ever been registered
Common name on the vial	Often labeled “CJC-1295 DAC” or “CJC-1295 with DAC”	Often labeled “CJC-1295 (no DAC)” or “Mod GRF 1-29”

What people use it for

USED FOR	EVIDENCE	WHAT THAT MEANS
Raising growth hormone and IGF-1 levels	**Human trials**	Established for the with-DAC form only, in a few dozen healthy volunteers; whether raising these levels produces any noticeable benefit is a separate, unanswered question.

USED FOR	EVIDENCE	WHAT THAT MEANS
Body composition, fat loss, or recovery	**Theoretical**	The one trial designed to test this, in people with HIV-associated abdominal fat gain, was terminated early and its results were never published.
Any use at all, in the without-DAC form	**Not studied**	No controlled human study of the without-DAC form exists for any purpose, and its half-life in humans has never been measured.
Combined with ipamorelin or other growth hormone secretagogues	**Theoretical**	No published human study has tested this combination; the pairing appears in several blend products but rests on an untested assumption that combining two release pathways adds benefit.

How it works

The pituitary gland normally releases growth hormone in pulses, prompted by a natural hormone signal from the brain. CJC-1295 is a modified copy of that signal, designed to bind the same receptor on the pituitary and trigger a pulse of growth hormone release, which then travels to the liver and prompts it to produce IGF-1.

The with-DAC form carries an added chemical group that forms a permanent bond with a protein circulating in the blood, which is what extends its activity to roughly a week per dose instead of minutes. The one study that measured the actual pattern of hormone release found that the pulses themselves were essentially unchanged in frequency or size — what rose was the baseline level of growth hormone between pulses, held elevated for days at a time, a different pattern than the body's own release.

Like other compounds in this class, it only works if the pituitary gland still has growth hormone available to release. If the pituitary itself is damaged, missing, or unresponsive, CJC-1295 has nothing to stimulate.

What the research actually looked at

The published human evidence for CJC-1295 is small and belongs almost entirely to the with-DAC form: three published studies, all in healthy volunteers, together amounting to a few dozen people, the longest lasting under two months, all measuring hormone levels rather than whether anyone actually got better.

The one trial designed to test whether the drug produces a real-world benefit — in people with HIV-associated abdominal fat gain — was terminated early after a participant had a fatal heart attack during the study. The treating physician attributed the event to pre-existing coronary artery disease rather than the drug, but the trial's results were never published, so the question it was designed to answer remains unanswered.

For the without-DAC form, sold in many blends today, there is no controlled human study of any kind — no efficacy data and no measured half-life. A 2024 regulatory review found no evidence of effectiveness in humans for any proposed use across all submitted forms, and flagged laboratory findings of DNA damage in pituitary cells in nonclinical testing.

Common misunderstandings

- **“CJC-1295 has a 6-to-8-day half-life.”** True only of the with-DAC form. If a vial is labeled without DAC (or “Mod GRF 1-29”), that half-life does not apply, and the actual figure has never been measured in a human study.

- **“It restores natural, pulsatile growth hormone release.”** The one study that measured this found pulse frequency and size were essentially unchanged — what rose was the baseline level between pulses, elevated for days at a time, a different pattern than natural release.
- **“It's well studied — there are published human trials.”** There are three, all in healthy volunteers, all using the with-DAC form, the longest lasting under two months, and all measuring hormone levels rather than clinical outcomes.
- **“It's basically approved since regulators reviewed it in 2024.”** Regulators proposed that none of the submitted forms be added to the approved compounding list, citing missing effectiveness data and safety signals in laboratory testing — a proposal against inclusion, not an approval.

Who it is not for

- Anyone with known coronary artery disease, a prior heart attack, or significant cardiac risk factors — the one efficacy trial ever run for this compound was stopped after a participant had a fatal heart attack.
- Anyone with active cancer, a cancer history, or an unscreened suspicious finding.
- Anyone with diabetes, prediabetes, or unstable blood sugar.
- Anyone with a known or suspected pituitary abnormality.
- Pregnant or breastfeeding patients, or anyone trying to conceive.
- Anyone whose supplier cannot say definitively whether the vial is the with-DAC or without-DAC form.

Competitive athletes: Prohibited at all times, named explicitly on the WADA Prohibited List alongside other growth hormone-releasing factors. Anti-doping testing can distinguish the with-DAC and without-DAC forms as separate substances, and a therapeutic use exemption is not a realistic option since there is no approved therapeutic use for either form.

Who makes this, and under what rules

Everything dispensed here is prepared by a licensed 503A compounding pharmacy — not bought from a supplier and repackaged, and not sourced from an online seller.

That designation carries specific, enforceable obligations:

- **A prescription in your name.** Compounded from a prescription for one identified patient, after an evaluation.
- **Licensed and inspected.** A state licence, subject to inspection by the state board of pharmacy.
- **Ingredients that can be traced.** A certificate of analysis from an FDA-registered facility, back to a named source and lot.
- **Sterile preparation to a published standard.** USP standards.
- **A paper trail.** A lot number and beyond-use date on every preparation.

A compounded preparation is not an FDA-approved medicine — but it is the difference between a licensed pharmacy working to a standard and an unregulated website.

Where this stands

Not approved, anywhere, in either form, in any country, for any use. Not on FDA's approved compounding list; it appears instead on a separate list of substances nominated and then withdrawn. FDA's December 2024 review proposed all five submitted forms not be included, citing no quality standard, missing purity and contamination data, and laboratory findings of DNA damage in pituitary cells.

A few dozen people have been studied. Three published studies, all using the anchored DAC version. The one trial designed to test whether it actually helps enrolled 120 people, was terminated early in 2006, and its results were never published.

That is not the same as saying it does nothing. It means the question has not been answered.

Clinical detail

Sold under one name but chemically two different molecules: with DAC (carrying an added albumin-binding anchor) and without DAC (also called Mod GRF 1-29), which clears within minutes.

The with-DAC form's measured half-life in human studies is roughly 6 to 8 days; the without-DAC form's half-life has never been reported in a published human study.

The only trial ever registered to test efficacy — in HIV-associated visceral obesity, 120 participants — was terminated in 2006 after a participant died during the study; results were never published.

Not on FDA's 503A compounding bulks list; sits instead in FDA's “nominated but withdrawn” table, with a December 2024 review proposing exclusion of all five submitted forms.

Frequently sold in combination blends with other growth hormone secretagogues such as ipamorelin, a pairing with no published human study of its own.

The information on this page is general education. It is not medical advice and it is not a recommendation for any treatment.

CJC-1295 is not an FDA-approved medicine for the uses described here. A compounded preparation is not an FDA-approved product and has not been reviewed by the FDA for safety, effectiveness or quality.

Descriptions of what this is used for reflect how it is commonly used and discussed. They are not claims that it is effective for any purpose.

Any prescription follows an individual evaluation by a licensed provider and is issued only where that provider judges it clinically appropriate. A prescription is never guaranteed.